

Announcement

Kindly note: We're currently experiencing a high volume of inquiries and calls, potentially leading to delays in our initial response.



Dermatology 23S Module 2

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Day 3 scenario 2

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[◀ Day 5 Scenario 3](#)[Day 2 Follow on Questions ▶](#) Display replies in nested form**Day 3 scenario 2**

by [Derek Kayanja \(Tutor\)](#) - Tuesday, 21 November 2023, 2:37 PM

Hi group,

Here is scenario 2.

A 68-year-old man presents with a recurrent bleeding wound on the lower lip of six weeks' duration. On questioning, he tells you that his work as a marketing agent involves spending a lot of time outdoors.

Examination reveals that there are no abnormalities except for an ulcer on the lower-external lip with induration, which fails to heal and bleeds recurrently. The ulcer feels firm when pressed between the finger and thumb.

The lesion:



Please answer all the questions below:

1. What is the clinical diagnosis on the basis of the lesion's characteristics?
2. What is the patient's level of risk for developing metastasis based on the clinical features?
3. What is the first therapeutic choice for the patient's condition?

122 words

Reply



Re: Day 3 scenario 2

by [Siddharth Saluja](#) - Tuesday, 21 November 2023, 5:03 PM

1) Based on the provided context, the clinical diagnosis based on the characteristics of the lesion is suggestive of a squamous cell carcinoma of the lower lip.

2) Regarding the patient's level of risk for developing metastasis based on the clinical features, it is important to consider that the presence of induration, failure to heal, and recurrent bleeding are concerning signs. However, a definitive assessment of metastatic risk requires further evaluation, such as histopathological examination and staging.(1)

3) The first therapeutic choice for the patient's condition would typically involve surgical excision of the lesion, (2) followed by appropriate management based on the stage and extent of the cancer. It is important for the patient to consult with a healthcare professional for a comprehensive evaluation and personalized treatment plan.

References:

1)Cleveland Clinic. (2022). Squamous Cell Carcinoma: Symptoms, Causes & Treatment. [online] Available at: <https://my.clevelandclinic.org/health/diseases/17480-squamous-cell-carcinoma>.

2)www.aad.org. (n.d.). Skin cancer types: Squamous cell carcinoma treatment. [online] Available at: <https://www.aad.org/public/diseases/skin-cancer/types/common/scc/treatment>.

155 words

Reply



Re: Day 3 scenario 2

by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:02 PM

Thank you Siddharth Good post correct diagnosis.

7 words

**Re: Day 3 scenario 2**

by [Pooran Outar](#) - Tuesday, 21 November 2023, 10:41 PM

(1) This lesion resembles a raised, dull-red skin lesion with an ulcerated appearance (raised edges and a lower area in the middle that might bleed), suggesting a probable **diagnosis of Squamous Cell Carcinoma of the Lower Lip** (Brown, 2014), (Gardner, 2022).

This patient present 4 of the following clinically important risk factors for developing SCC according to the American Cancer Society, 2023:

- UV light exposure
- Having light-coloured skin
- Age > 50 years
- Male gender
- Exposure to certain chemicals and radiation
- Long-term or severe skin inflammation or injury
- Inherited conditions like Xeroderma Pigmentosum
- Other genetic syndromes linked with an increased risk of skin cancer

(2) As per the guidelines outlined by the Scottish Intercollegiate Guidelines Network, the following features are classified as high-risk indicators for SCC (Wells, 2021), (Oakley, 2015):

- Elderly or Immunosuppressed
- Tumor located in the ear, vermillion of the lip, central face, hands, feet, genitalia
- Horizontal tumor diameter exceeding 20 mm
- Tumor depth greater than 4 mm, with a threshold of >6 mm
- Tumor extension beyond the dermis into or through subcutaneous fat
- Perineural invasion
- Desmoplastic subtype
- Poorly differentiated tumor status

Considering the presence of two of these high-risk features, **the overall risk for metastasis in this patient is deemed high.**

(3) Considering the factors above then **Excision of the lesion with a safety margin** would be the therapeutic option of choice in this case (Hale and Hanke, 2022).

References:

- American Cancer Society (2023). *Basal and Squamous Cell Skin Cancer Risk Factors | Skin Cancer Risks*. [online] [www.cancer.org](https://www.cancer.org/cancer/types/basal-and-squamous-cell-skin-cancer/causes-risks-prevention/risk-factors.html). Available at: <https://www.cancer.org/cancer/types/basal-and-squamous-cell-skin-cancer/causes-risks-prevention/risk-factors.html>.
- Brown, T. (2014). *Squamous Cell Carcinoma*. [online] WebMD. Available at: <https://www.webmd.com/melanoma-skin-cancer/squamous-cell-carcinoma#1>.
- Gardner, S.S. (2022). *Basal Cell and Squamous Cell Carcinoma*. [online] ucsfhealth.org. Available at: <https://www.ucsfhealth.org/conditions/basal-cell-and-squamous-cell-carcinoma>.
- Hale, E.K. and Hanke, W. (2022). *Squamous Cell Carcinoma Treatment*. [online] The Skin Cancer Foundation. Available at: <https://www.skincancer.org/skin-cancer-information/squamous-cell-carcinoma/scc-treatment-options/>.
- Oakley, A. (2015). *Cutaneous squamous cell carcinoma | DermNet NZ*. [online] [dermnetnz.org](https://dermnetnz.org/topics/cutaneous-squamous-cell-carcinoma). Available at: <https://dermnetnz.org/topics/cutaneous-squamous-cell-carcinoma>.
- Wells, J.W. (2021). *Cutaneous Squamous Cell Carcinoma Guidelines: Guidelines Summary*. [online] [emedicine.medscape.com](https://emedicine.medscape.com/article/1965430-guidelines#g1). Available at: <https://emedicine.medscape.com/article/1965430-guidelines#g1>.

328 words

[Reply](#)**Re: Day 3 scenario 2**by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:02 PM

Thank you Pooran, good post correct diagnosis.

7 words

[Reply](#)**Re: Day 3 scenario 2**by [Mostafa Damghani](#) - Sunday, 26 November 2023, 7:12 PM

Thank you Pooran for the your informative post, just a question on my side, as I was studying the Scottish Intercollegiate Guidelines Network for the high risk features of SCC.

there are two sets of features, one based on good evidence and one based on limited evidence, would there be a difference to approach a patient's management who is at high risk due to presence of one over the other? How do you take this information into context into clinical practice?

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81 words

[Reply](#)**Re: Day 3 scenario 2**by [Sophie Holloran](#) - Wednesday, 22 November 2023, 7:34 AM

1. What is the clinical diagnosis based on the lesion's characteristics?

This is a fairly small, ulcerated lesion on the lower lip that is just inside the vermillion border. It is non healing and bleeds regularly.

From the image and the history I would be concerned that this is a **cutaneous squamous cell carcinoma**.

This patient spends a lot of time outdoors which would increase his risk. It looks like he has quite poor dentition and I would want to know if he was a smoker? Smoking has been reported to increase the risk of cSCC (Arafa 2020).

To confirm my suspicion I would want to examine the lesion with dermoscopy.

2. What is the patient's level of risk for developing metastases based on the clinical features?

This table summarises the features of high risk cSCC that have been associated with an increased risk of metastases (Veness 2007):

Table 1

High-risk factors (patients often have multiple high-risk factors present).

	Factor
(1)	Large size (>2 cm).
(2)	Thick or deeply invasive lesion (>4 mm).
(3)	Incomplete excision (<4 mm).
(4)	Recurrent setting.
(5)	High-grade or desmoplastic lesion.
(6)	Presence of perineural invasion.
(7)	Presence of lymphovascular invasion.
(8)	Located near the parotid gland (ear, temple, forehead, ant. scalp).
(9)	Immunosuppressed state (e.g., transplant recipient).

DermNet lists the following features for high risk cSCC (Oakley 2015):

- Diameter greater than or equal to 2 cm
- Location on the ear, vermilion of the lip, central face, hands, feet, genitalia
- Arising in elderly or immune suppressed patient
- Histological thickness greater than 2 mm, poorly differentiated histology, or with the invasion of the subcutaneous tissue, nerves and blood vessels.

The high risk features for this patient would be its location on the lip and its feeling of thickness, although this would need to be confirmed on histology. It also looks from the image like this lesion is <2cm in size. I would say from clinical examination from alone and from the history this appears to be a relatively low risk lesion, but it would need to be fully excised to assess some of the other potential high risk factors.

3. What is the first therapeutic choice for the patient's condition?

The gold standard approach would be to surgically excise this lesion with a margin of at least 5mm.

Question for the Group:

Are there any additional/specific factors that need to be considered regarding surgical excision, given that this lesion is on the lip?

If the patient declined to have a surgical excision, what would your next steps be?

References:

1. Arafa A, Mostafa A, Navarini AA, Dong JY. (2020) The association between smoking and risk of skin cancer: a meta-analysis of cohort studies. Cancer Causes Control. Aug;31(8):787-794. doi: 10.1007/s10552-020-01319-8. Epub 2020 May 27. PMID: 32458137.

2. Oakley (2015) *Cutaneous Squamous Cell Carcinoma* - DermNet NZ. Available at: <https://dermnetnz.org/topics/cutaneous-squamous-cell-carcinoma> (Accessed 22/11/23)
3. Veness MJ. High-risk cutaneous squamous cell carcinoma of the head and neck. *J Biomed Biotechnol.* 2007;2007(3):80572. doi: 10.1155/2007/80572. Epub 2007 Apr 3. PMID: 17541471; PMCID: PMC1874675.
4. British Society of Dermatological Surgery - Surgical Management of Non-melanoma Skin Cancers. Available at: <https://bsds.org.uk/members/workshop/surgical-management-of-non-melanoma-skin-cancer/> (Accessed 22/11/23)

456 words

Reply



Re: Day 3 scenario 2

by [Xiao Liey Ding](#) - Wednesday, 22 November 2023, 11:32 AM

Hi Sophie,

Thank you for the questions.

Question 1.

There are specific considerations for surgical excision due to the unique anatomy and cosmetic importance of the lip. The lip is a highly visible and functional part of the face. Surgical planning should aim to achieve optimal cosmetic and functional results, considering the potential impact on speech, eating, and facial aesthetic. Given the cosmetic significance of the lip, reconstruction methods should be considered during surgical planning. This may involve techniques such as primary closure, local flaps, or grafts to restore both form and function (Sung, B.Y. *et al*, 2023).

Question 2.

If a patient declines surgical excision, my next step would be to thoroughly discuss the potential risks and consequences with patient. Provide patient with detailed information about the risks of leaving the lesion untreated, including the potential for local progression, invasion into surrounding tissues, and the increased risk of metastasis. Establish a close monitoring plan with the patient, including regular follow-up appointments to assess the lesion's progression or any changes. This may involve clinical examinations, imaging studies, or other diagnostic tests. Ensure that the patient fully understands the implications of declining surgery and obtains informed consent for their decision. Document these discussions and the patient's decision in their medical records. Involve other healthcare professionals, such as oncologists or dermatologists, to discuss alternative treatment options and potential consequences. I would also offer psychosocial support to address the patient's concerns and fears.

Reference:

Sung, B.Y., Hoon, M., and Ik-Jae, K. (2023). Squamous Cell Carcinoma of Lower Lip: The Results of Wide V-shaped Resection. *J Korean Assoc Oral Maxillofac Surg*,49(5):292-296. Doi: 10.5125/jkaoms.2023.49.5.292 (Accessed 22/11/2023)

272 words

Reply



Re: Day 3 scenario 2

by [Siddharth Saluja](#) - Wednesday, 22 November 2023, 1:13 PM

Hi Sophie,
In answer to your questions:

1) When considering surgical excision of a basal cell carcinoma on the lip, there are a few additional factors that need to be considered: (1)

1. Functional and cosmetic outcomes: The lip is a highly visible and functional area, so preserving both function and aesthetics is crucial. The size and location of the lesion will influence the surgical approach to minimize any potential functional or cosmetic impairments. (1)

2. Reconstruction options: Depending on the size and extent of the excision, reconstructive techniques may be necessary to restore the lip's form and function. This may involve primary closure, local flaps, or grafts. (1)

3. Margin control: Basal cell carcinomas require complete removal with clear margins to minimize the risk of recurrence. Achieving adequate margins while preserving lip function and aesthetics can be challenging and may require careful planning and coordination with a skilled surgeon. (2)

2) If the patient declines surgical excision of the basal cell carcinoma, it is important to have a detailed discussion with the patient regarding the risks and benefits of alternative treatment options. These may include non-surgical approaches such as topical medications (e.g., imiquimod), cryotherapy, or radiation therapy. However, it is crucial to emphasize that the effectiveness of these alternatives may vary, and surgical excision remains the gold standard for complete removal of the lesion. (3)

Ultimately, the next steps would depend on the patient's preferences and the recommendations of a healthcare professional. It is important to ensure that the patient is well-informed about the potential consequences of declining surgical excision and to explore alternative treatment options that align with their preferences and medical needs.

References:

1) Paoli, J., Dahlén Gyllencreutz, J., Fougberg, J., Johansson Backman, E., Modin, M., Polesie, S. and Zaar, O. (2019). Nonsurgical Options for the Treatment of Basal Cell Carcinoma. *Dermatology Practical & Conceptual*, pp.75–81. doi:<https://doi.org/10.5826/dpc.0902a01>.

2) Lien, M.H. and Sondak, V.K. (2011). Nonsurgical Treatment Options for Basal Cell Carcinoma. *Journal of Skin Cancer*, 2011, pp.1–6. doi:<https://doi.org/10.1155/2011/571734>.

3) Authority (2023). Exploring the Alternatives to Mohs Surgery. [online] *Dermatology and Skin Health* - Dr. Mendese. Available at: <https://dermskinhealth.com/exploring-the-alternatives-to-mohs-surgery/> [Accessed 22 Nov. 2023].

357 words

Reply



Re: Day 3 scenario 2

by [Sandeep Kumar](#) - Wednesday, 22 November 2023, 2:04 PM

Hi Sid,
I agree with you that the treatment would depend on patient's preferences as this is a very sensitive region to go aggressive with the treatment and some patients might decide to leave it alone given slow spread and looking at post-op pictures from other patients.

47 words

Reply



Re: Day 3 scenario 2

by [Rahima Abdulaziz Ahmed](#) - Wednesday, 22 November 2023, 4:10 PM

Thank you for the good discussion colleagues. To answer your question Sophie about the factors to consider regarding surgical excision of the lip squamous cell carcinoma, its important for us to understand first and foremost that the lips play an important role in our communication, oral functions and facial esthetics. (Youn, Kwon and Myoung 2023)

Factors to consider: (Youn, Kwon and Myoung 2023)

1. Lip Anatomy: given the unique anatomy of the lower lip, preservation of both form and function is critical. Techniques such as Mohs micrographic surgery or complex closures may be considered to optimize cosmetic and functional outcomes.

2. Size and depth of the lesion: The size and depth of the lesion SCC influence the extent of excision needed. Deep lesions or those involving critical structures may require more extensive surgery.

3. Lymph node Evaluation: Assessment of regional lymph nodes may be necessary, especially if the SCC is aggressive or has a higher risk of metastasis. Sentinel lymph node biopsy or imaging studies may be considered.

4. Patient factors: consider overall health and comorbidities of the patient. Collaboration with other specialists e.g. oral maxillofacial surgeons, anesthesiologists, cardiologists may be necessary.

5. Histopathological Features: A detailed examination of the histopathological features of the SCC is crucial for assessing its aggressiveness and potential for metastasis.

6. Margins: Adequate margins are essential to ensure complete removal of the tumor. Clear margins reduce the risks of recurrence.

7. Reconstruction Options: Depending on the extent of the excision, reconstructive options such as primary closure, local flaps or grafts may be considered to restore both aesthetics and function.

References:

Youn, B. Myoung, H and Kwon, J. (2023) 'Squamous cell carcinoma of lower lip: the results of wide V-shaped resection' *Journal of The Korean Association of Oral and Maxillofacial surgeons* Published online (doi: 10.5125/jkaoms.2023.49.5.292, 49(5): 292–296.) Available at

:<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC10618663/> (Accessed on 22/11/23)

305 words

Reply



Re: Day 3 scenario 2

by [Sophie Holloran](#) - Wednesday, 22 November 2023, 7:34 PM

Thanks for your answers, I don't think I could have summarised it better myself! I think it would be quite daunting to have a procedure done to your lip, for all the reasons you have all mentioned. I agree with all of your approaches if the patient declined the excision, I think I would do the same.

57 words

Reply



Re: Day 3 scenario 2

by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:03 PM

Thank you Sophie, very good post and nice table.

9 words

Reply



Re: Day 3 scenario 2

by [Arwaf Benrawwaf](#) - Friday, 15 December 2023, 2:27 PM

Hello Sophie, thank you for your post. According to a study published in the Journal of Oral and Maxillofacial Surgery, recommends caution when performing surgical excision of lip lesions to preserve function and minimize scarring. A modified elliptical incision technique is suggested to reduce tension on wound edges and preserve the natural contour of the lip.

If a patient declines surgery, further evaluation and management may be necessary depending on the nature of the lesion. The American Academy of Dermatology recommends performing a biopsy to determine the nature of suspicious lesions and considering alternative treatments for malignancy. Close monitoring with regular follow-up appointments is recommended for benign lesions.

References:

-Gomaa, M. and Samir Halawa (2019). Solitary angiokeratoma of the tongue: Report of a rare case. *Journal of Oral and Maxillofacial Surgery, Medicine, and Pathology*, 31(1), pp.43–46.
doi:<https://doi.org/10.1016/j.ajoms.2018.08.003>.
-American Academy of Dermatology Association (n.d.). *Types of skin cancer*. [online] www.aad.org. Available at: <https://www.aad.org/public/diseases/skin-cancer/types/common>.

153 words

Reply



Re: Day 3 scenario 2

by [Sandeep Kumar](#) - Wednesday, 22 November 2023, 2:02 PM

Question 1

According to the presentation - recurrent bleeding, duration six weeks, time spent outdoors and examination showing induration and non-healing ulcer with recurrent bleeding point out towards the diagnosis of squamous cell carcinoma of lower lip.

Question 2.

The squamous cell carcinoma of the lips are low cancers on an average five years survival rate is around 90% with risk of metastasis, increasing according to the stage of cancer.
The lesions located near the lymph nodes are more likely to spread.
The patients with comorbidities are more likely to develop metastasis
Most commonly metastasize to cervical lymph nodes with the study, mentioned in reference finding 12% metastasis for T1 stage SCC.

Question 3

If detected early, the cure rates are upwards of 90% with surgery or radiotherapy alone.
Unfortunately if there is cervical metastasis, then average five years survival rate would be around 50%.
The treatment will be guided by staging and note involved if there is spread, then more aggressive excision along with Neck dissection is also recommended.

Trivia- 95% of SCC on lips happen in Vermilion region. I had to search which part is 'Vermilion of lips'.

References:

Bon-Mardion, N., de Raucourt D, Babin, E., Rame Jp, D. Dehesdin and O. Choussy (2015). Cervical lymph node metastases and T1 squamous cell carcinoma of the lips. *PubMed*, 11(2), pp.89–93.

Kornevs E, Skagers A, Tars J, Bigestans A, Lauskis G, Libermanis O. 5 year experience with lower lip cancer. *Stomatologija*. 2005;7(3):95-8. PMID: 16340275.

Dediol E, Luksić I, Virag M. Treatment of squamous cell carcinoma of the lip. *Coll Antropol*. 2008 Oct;32 Suppl 2:199-202. PMID: 19140283.

Bilkay U, Kerem H, Ozek C, Gundogan H, Guner U, Gurler T, Akin Y. Management of lower lip cancer: a retrospective analysis of 118 patients and review of the literature. *Ann Plast Surg*. 2003 Jan;50(1):43-50. doi: 10.1097/01.SAP.0000024740.66871.BF. PMID: 12545108.

306 words

Reply



Re: Day 3 scenario 2

by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:04 PM

Thank you Sandeep, good post

5 words

[Reply](#)



Re: Day 3 scenario 2

by [Derek Kayanja \(Tutor\)](#) - Wednesday, 22 November 2023, 5:22 PM

Hi group, thank you everybody who has posted on this scenario, you are all on the right track, I shall make some comments tomorrow when a few more people have had a chance to respond.

35 words

[Reply](#)

**Re: Day 3 scenario 2**by [Oyeombiliyetu Jason](#) - Wednesday, 22 November 2023, 10:51 PM

1. Squamous cell carcinoma, due to:

Light(white) skinned

Male

Age: >50

Occupation: an extended period of sun exposure

Lesion: persistent ulcer, on the external lower lip, with induration, with recurrent bleeding, not healing.

2. Risk for developing metastasis

According to the Scottish intercollegiate guideline network, SCC should be divided into low and high-risk. With the latter having a greater risk of metastasis. The risks are as follows:

Sex: **M>F**

Age: **>50**

Immune status

Tumour site: ear, nose, **cutaneous lips**, eyelids, scalp.

Special clinical situation: SCC arising from site of trauma, burn, scar, pre-existing skin lesion.

Size of lesion: >2cm

Tumor depth: >4mm

Perineural invasion

Lymphovascular invasion

Tumor subtype: desmoplastic subtype.

Differentiation: poorly differentiated

Incomplete excision: positive margins

A patient that exhibits any of the above criteria is considered high risk.

3. Once the patient meets the criteria for high-risk lesion

multidisciplinary team gets involved

Mohs micrographic surgery would be ideal, for this lesion at this anatomical site, if not possible to preserve the anatomy, primary or adjuvant radiotherapy can be an alternative.

Patients need to follow up every 3-6 months for the first 24 months posttreatment, then yearly for the following 3 years.

And most importantly continuous sun protection.

References

1. J. Howell(2023). 'Squamous cell skin cancer'. National Library of Medicine. Available at:

<https://www.ncbi.nlm.nih.gov/books/NBK441939/> accessed: 22 November 2023

2. J. Well(2021). 'Cutaneous squamous cell carcinoma'. Medscape. Available at:

[https://emedicine.medscape.com/article/1965430-overview?](https://emedicine.medscape.com/article/1965430-overview?scode=msp&st=fpf&socialSite=apple&icd=login_success_ap_match_fpf&form=fpf#a1)

[scode=msp&st=fpf&socialSite=apple&icd=login_success_ap_match_fpf&form=fpf#a1](https://emedicine.medscape.com/article/1965430-overview?scode=msp&st=fpf&socialSite=apple&icd=login_success_ap_match_fpf&form=fpf#a1) accessed: 22 November 2023.

233 words

Reply

**Re: Day 3 scenario 2**by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:05 PM

Thank you Oyeombiliyetu, good post

5 words

Reply

**Re: Day 3 scenario 2**by [Derek Kayanja \(Tutor\)](#) - Friday, 24 November 2023, 12:11 PM

Hi Group,

Very well done all !.

Have you have correctly stated this is a case of Squamous cell carcinoma, this patient's level of risk for developing metastasis based on the clinical features is High. And the first therapeutic choice for the patient's condition is excision.

Squamous cell carcinoma (SCC) usually occurs on sites exposed to the sun, such as the backs of the hands and forearms, the upper part of the face and, especially in males, on the lower lip and pinna. The first clinical evidence of malignancy is induration. The lesion invariably feels firm when pressed between the finger and thumb. On mobile structures such as the lip or genitalia, SCC may present as a fissure or small erosion or ulcer, which does not heal and bleeds recurrently. This lesion presents a high risk for metastasis. For this reason, surgical excision should be the first choice of treatment.

150 words

Reply



Re: Day 3 scenario 2

by [Mostafa Damghani](#) - Sunday, 26 November 2023, 6:43 PM

Based on the symptoms the diagnosis is SCC:

- Firm lesion= **Induration**
- Lower lips of six weeks =**sun-exposed** area and developed in weeks
- Spending a lot of time outdoors **increased exposure to the sun.**
- persistent **lesion failing to heal with recurrent bleeding**=typical features in SCC.

Risk of Metastasis in this patient:

Squamous cell carcinoma has a low rate of metastasis at around 5%, SCC that display high-risk features can increase the risk of metastasis in certain patients. Scottish Intercollegiate Guidelines Network (SIGN) helps evaluate the risk of metastasis in local lesions.

High-risk features based on good evidence:

Clinical features:

- Tumour diameter of more than 2 cm
- Lesions on the ear
- Immunosuppression
- Recurrent SCC

Pathological features:

- Tumour thickness of more than 4mm
- Tumour extension beyond the dermis into the subcutaneous fat
- Perineural invasion
- Poorly differentiated
- Desmoplastic subtype

Therapeutic choice

The first choice for this patient would be **surgical excision** of the tumour to prevent metastasis as this patient is at a higher risk due to possible induration, at the location of the lesion on the lip.

References:

1-Scottish Intercollegiate Guidelines Network (SIGN) . Management of primary cutaneous squamous cell carcinoma . Edinburgh : SIGN , 2014 , SIGN publication no. 140. Cited 16 June 2014

2-dermnetnz.org. (n.d.). *Cutaneous squamous cell carcinoma* / *DermNet NZ*. [online] Available at: <https://dermnetnz.org/topics/cutaneous-squamous-cell-carcinoma>.

3-Rook, A., Griffiths, C.E.M. and Al, E. (2016). *Rook's Textbook of dermatology*. West Sussex: Wiley Blackwell chapter 142-29 treatment of squamous cell carcinoma.



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252 words

Reply

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